DISEASE BACKGROUND

# Schizophrenia and bipolar disorder

Clinical background on the two conditions in which olanzapine is used. Product information, evidence and prescribing details are in the sections that follow.

Schizophrenia | Bipolar disorder

## Schizophrenia

Schizophrenia is a psychiatric disorder that can affect how a person perceives reality, thinks, experiences emotions and interacts with others. Its presentation varies between individuals, and symptoms can change over time.

Treatment aims to manage symptoms, prevent relapse where possible, support everyday functioning and address the individual's broader healthcare needs.

# Understanding the symptom domains

## 01 Positive symptoms

Experiences or behaviours that are **added** to a person's usual functioning.

- Hallucinations
- Delusions
- Disorganized thinking or speech
- Disorganized behaviour

These symptoms may substantially affect a person's perception of reality and ability to function.

## 02 Negative symptoms

A **reduction or loss** of aspects of normal functioning. Examples may include:

- Reduced motivation
- Diminished emotional expression
- Reduced speech
- Social withdrawal
- Reduced engagement in everyday activities

Negative symptoms can affect social relationships, occupational functioning and participation in daily life.

## 03 Cognitive symptoms

Difficulties with the **mental processes** needed for everyday activities. These may include difficulties with:

- Attention and concentration
- Working memory
- Planning and organization
- Decision-making and executive functioning

The pattern and severity of symptoms differ between individuals. Cognitive difficulties should be considered as part of the overall clinical assessment.

Three symptom domains of schizophrenia, with examples. Domains are not mutually exclusive and individual presentations vary.[4]

# The neurobiology of schizophrenia

Schizophrenia is associated with complex changes in brain function and neurotransmission. One widely discussed model links three interconnected hypotheses: excess dopamine $D_2$ activity in the mesolimbic pathway, hypofunction of glutamate NMDA receptors, and hyperfunction of cortical serotonin 5-$HT_{2A}$ receptors.[5]

In this model, NMDA receptor hypofunction and 5-$HT_{2A}$ hyperfunction can each lead, further downstream, to mesolimbic dopamine hyperactivity, which is associated with positive symptoms. Negative and cognitive symptoms are thought to involve other circuits.[5] No single neurotransmitter imbalance fully explains schizophrenia or accounts for every symptom an individual experiences.

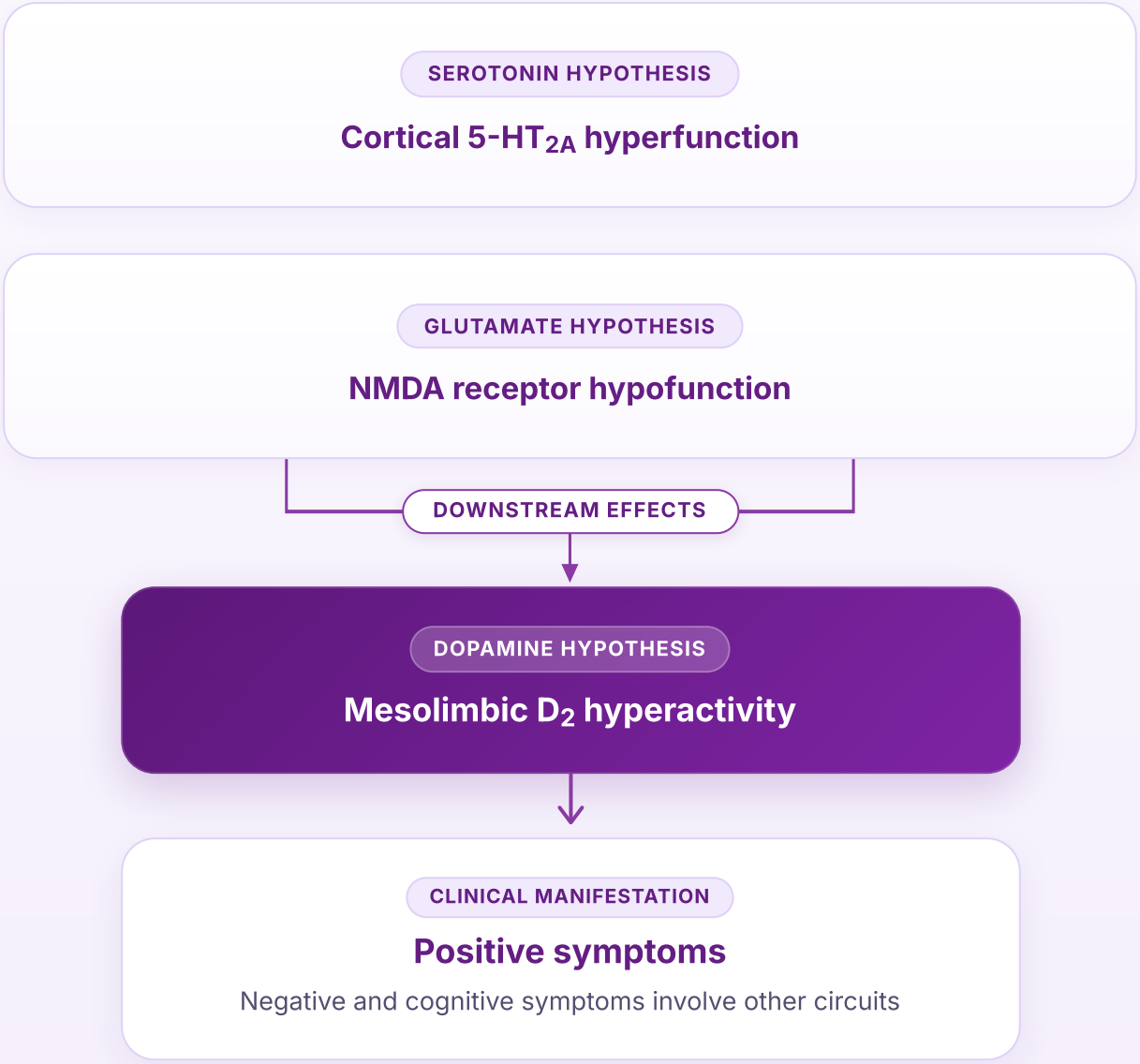
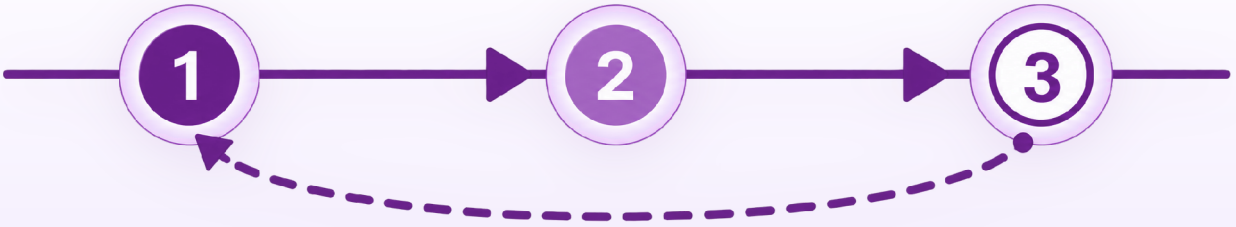



Figure 1. Simplified schematic of the dopamine, glutamate and serotonin hypotheses and their proposed links. It does not depict anatomy or individual neurons. Adapted from Stahl 2018.[5]

# Treatment across the course of schizophrenia

Treatment needs can change over time. The following phases are a useful clinical framework rather than a rigid sequence that applies identically to every patient.




01

PHASE

### Acute treatment

The immediate objectives are to assess clinical severity and safety, manage acute symptoms and establish an appropriate treatment plan.

02

PHASE

### Stabilization

As symptoms improve, treatment focuses on consolidating the response, reviewing tolerability and supporting a return to everyday routines.

03

PHASE

### Maintenance

Ongoing treatment and follow-up aim to sustain clinical improvement, monitor adverse effects and identify early signs of deterioration.

Figure 2. Phases of care as a clinical framework. Progression is not fixed, and individual patients may move between phases. Treatment needs can change over time; phases may recur.

### Supporting continuity of care

Follow-up should consider treatment response, tolerability, daily functioning, social support and the individual's ability to continue the agreed treatment plan.

Adherence may be affected by adverse effects, the nature of the illness, treatment preferences and practical barriers. Explore these collaboratively rather than attributing them to the patient alone.

## Bipolar disorder

Bipolar disorder is characterized by episodes of mood disturbance, including mania or hypomania and depression, depending on the type of bipolar disorder. Episodes can affect energy, sleep, judgement, behaviour and everyday functioning.

Treatment decisions depend on the current episode, previous clinical history, treatment response, comorbidities and the individual's needs.

# Understanding mood episodes

ELEVATED, EXPANSIVE OR IRRITABLE MOOD

01 **Mania**

Mania involves a distinct period of abnormally elevated, expansive or irritable mood and increased activity or energy. Symptoms may include reduced need for sleep, increased speech, racing thoughts, inflated self-esteem and impulsive or high-risk behaviour.

Severe manic episodes may be associated with marked functional impairment and can require urgent clinical assessment.

FEATURES OF BOTH

02 **Mixed features**

Some episodes involve symptoms associated with both mania and depression. These presentations can be complex and require careful clinical assessment.

PERSISTENT LOW MOOD

03 **Depression**

Depressive episodes may involve persistent low mood, reduced interest or pleasure, fatigue, changes in sleep or appetite, feelings of worthlessness and difficulties with concentration.

The clinical presentation and treatment needs differ from those of mania.

Features of manic, mixed and depressive episodes.[4] The arrangement groups episode features; it does not represent an illness course, the order in which episodes occur, or their frequency.

# Treatment objectives

Treatment objectives depend on the phase of illness.

### Acute management

Address the current episode and associated clinical risks.

### Stabilization

Consolidate improvement and review tolerability.

### Recurrence prevention

Support sustained clinical stability following a treatment response.

These objectives should be individualized. Treatment for an acute manic episode should not automatically be assumed to provide adequate management for every subsequent mood episode.

### Product information

Approved indications, dosing and safety information for olanzapine are in the overview and in the dosing and safety sections.

**Approved indications →**

← Previous screen: **Overview**

Next screen: **Mechanism of action** →

## Address

CHEPLAPHARM Registration GmbH
Weiler Straße 5e
79540 Lörrach, Germany

## Contact

+49 7621 593 06-00

+49 7621 593 06-98

info@cheplapharm-reg.com


## Legal

Imprint

Data Privacy

Conditions of Use

This website contains information on ZYPREXA (olanzapine) coated tablets, which is based on the approved EU Summary of Product Characteristics (SmPC). It is intended for educational and reference purposes only for healthcare professionals. Always consult the current approved product information before prescribing. ZYPREXA® is a registered trademark of CHEPLAPHARM. Prepared October 2026. Job code to be assigned for the target market.

© 2026 CHEPLAPHARM Registration GmbH